LOGO

**[Practice name]**
[Street address] · [City, State / Region, Postcode]
[Phone] · [Email] · [Website]



CLINICAL NOTE + REPORT TO PRIMARY CARE / ENDOCRINOLOGY

# Diabetic Eye Examination & Report

Grade retinopathy and macular oedema per eye, build the ICD-10 code, set follow-up, and send page 2 to the managing provider to close the care loop.

Completed by: Clinician | Use as: Note template + letter | Time: 10–15 min

| Patient name | Date of birth | Patient ID / MRN | Date of service | Clinician |
|---|---|---|---|---|
| | | | | |

## 01 Diabetes profile

**Type**
○ Type 1 ○ Type 2 ○ Gestational ○ Other

**Years since diagnosis**

**Last A1c / date** %

**Blood pressure** mmHg

**Treatment**
☐ Diet ☐ Oral agents ☐ Insulin ☐ GLP-1 / GIP agonist ☐ Insulin pump / CGM

**Factors**
☐ Nephropathy ☐ Neuropathy ☐ Hyperlipidaemia ☐ Smoker ☐ Pregnant / planning ☐ Rapid A1c improvement (< 6 months)
☐ Previous laser / injections

**Managing provider (PCP / endocrinologist)**

**Phone / fax**

## 02 Examination

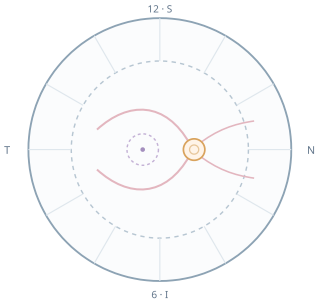
| Visual acuity | Distance SC | Distance CC | Pinhole | Near SC | Near CC |
|---|---|---|---|---|---|
| OD | | | | | |
| OS | | | | | |
| OU | | | | | |

| Anterior | IOP (mmHg) | Iris / angle NV | Lens | Dilated |
|---|---|---|---|---|
| OD | | ○ None ○ NVI ○ NVA | | ○ Yes ○ No |
| OS | | ○ None ○ NVI ○ NVA | | ○ Yes ○ No |

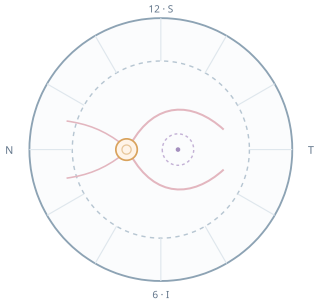
| Retina | DR severity | Macular oedema | OCT CST | Other findings |
|---|---|---|---|---|
| OD | ○ None ○ Mild NPDR ○ Mod NPDR ○ Severe NPDR ○ PDR ○ High-risk PDR | ○ None ○ Non-CI ○ CI-DME | µm | |
| OS | ○ None ○ Mild NPDR ○ Mod NPDR ○ Severe NPDR ○ PDR ○ High-risk PDR | ○ None ○ Non-CI ○ CI-DME | µm | |

**Mild NPDR** microaneurysms only · **Moderate** more than MAs but less than severe · **Severe (4-2-1):** > 20 intraretinal haemorrhages in each of 4 quadrants, OR venous beading in ≥ 2 quadrants, OR prominent IRMA in ≥ 1 quadrant · **PDR** NVD / NVE, vitreous or pre-retinal haemorrhage. **CI-DME** = oedema involving the central subfield.

Retina — OD OD




Retina — OS OS




## 03 Coding

| ICD-10 builder | Type (E10 / E11) | Severity digit | Oedema digit | Laterality (7th) | Full code |
|---|---|---|---|---|---|
| Code | | ○ 2 mild ○ 3 mod ○ 4 sev ○ 5 PDR | ○ 1 with ME<br>○ 9 without ME | ○ 1 R ○ 2 L<br>○ 3 Bilat | |

Structure: E11.**3[severity][oedema]-[eye]** — e.g. E11.3293 = type 2, mild NPDR without macular oedema, bilateral. No retinopathy: E11.9 / E10.9. Add Z79.4 (insulin) or Z79.84 (oral agents) as applicable. PDR also has 5th-character options for traction detachment (.352–.354) and stable PDR (.355).

- ☐ **2022F** Dilated retinal exam, interpreted & reviewed — retinopathy present
- ☐ **2023F** Dilated retinal exam, interpreted & reviewed — no retinopathy
- ☐ **2024F** 7-field stereo photos interpreted — retinopathy present
- ☐ **2025F** 7-field stereo photos interpreted — no retinopathy
- ☐ **2026F** Validated eye imaging — retinopathy present
- ☐ **2033F** Validated eye imaging — no retinopathy
- ☐ **5010F** Findings communicated to managing physician

## 04 Plan & follow-up

| Guide (AAO PPP) — verify current guidance | No ME | Non-CI DME | CI-DME |
|---|---|---|---|
| No apparent DR / mild NPDR | 12 months | 3–6 months | 1 month |
| Moderate NPDR | 6–12 months | 3–6 months | 1 month |
| Severe NPDR / non-high-risk PDR | 3–4 months | 2–4 months | 1 month |
| High-risk PDR | 2–4 months | 2–4 months | 1 month |

Plan

☐ Annual dilated exam ☐ Repeat sooner ☐ OCT macula ☐ Fundus photography ☐ Refer retina — routine ☐ Refer retina — urgent ☐ Anti-VEGF / laser (EYE-21)

Return in

Patient counselling (glucose, BP, lipids, smoking)

Clinician signature

×

Clinician name & credentials

Date / time signed

## 05 Report to managing provider

page can be sent as a letter

To (provider / clinic) | Fax / secure email | Date sent

Dear colleague, your patient Patient name (DOB DOB) had a dilated diabetic eye examination on Date of service. Findings are summarised below.

| Summary | Best-corrected VA | Retinopathy | Macular oedema | Other |
|---|---|---|---|---|
| OD | | | | |
| OS | | | | |

Overall

○ No diabetic retinopathy ○ Retinopathy present — see above ○ Referred to retina specialist

Recommendations

Next eye exam due

Good glycaemic, blood pressure and lipid control reduce the risk of progression. Please contact us with any questions.

Clinician name & credentials | Signature | Phone